Op. Dr. Murat Ayata
Kulak Burun Boğaz
Otolaryngology Head and Neck Surgery Specialist

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

INFORMED CONSENT FORM

Deep Neck Lift

Neck lift that also treats the deeper structures of the neck (deep-plane neck lift)

Dear patient, this form has been prepared to explain, in **plain and clear language**, what you need to know about the procedure recommended to you. Its purpose is not to frighten you, but to help you make your decision **freely and on the basis of good information**.

Please read this form without rushing. Ask your doctor about anything you do not understand. Your doctor will also explain verbally any risks that are particular to your situation. Whether or not you give consent is entirely your right.

PATIENT AND PROCEDURE DETAILS

Full name: _____ Date of birth: _____

Passport / ID No: _____ File / Protocol No: _____

Telephone: _____ Date informed: _____

Planned procedure date: _____

Form first sent to patient (date / e-mail, WhatsApp, in person): _____

Doctor performing the procedure and providing information: **Op. Dr. Murat Ayata**

Hospital / facility where the procedure will be performed: _____

Planned components: ☐ Platysmaplasty ☐ Deep fat removal ☐ Digastric reduction ☐ Submandibular gland reduction ☐ Skin excision

Simultaneous facelift: ☐ No ☐ Yes (with a separate consent form) _____

Other procedures to be performed at the same time: _____

Patient's concerns and expectations: _____

1 Your condition and the aim of the procedure

With age, weight changes or inherited features, the neck may develop **fullness under the chin**, loss of the angle between the jaw and the neck, **vertical bands** (bands of the platysma muscle) and **loose skin**. This appearance is often caused not only by fat under the skin, but also by **deep fat beneath the neck muscle, the muscles under the chin (digastric muscles) and sagging of the salivary glands (submandibular glands)**.

A deep neck lift addresses these deeper structures as well, with the aim of defining the jawline and achieving a **younger, sharper and natural-looking** neck. The result depends on your skin elasticity, bone structure, weight and age. The operation does not stop ageing; the neck will continue to relax gradually over time.

2 How is the procedure performed?

The operation is usually performed under general anaesthesia in an operating theatre. Beforehand, your neck is examined while you sit and stand, photographed and marked, and the structures to be treated are agreed with you.

1. **Incisions:** An incision of about 3–5 cm is made in the natural crease under the chin. If excess skin is to be removed, incisions are added around the earlobe, extending behind the ear towards the hairline. Incisions are hidden in natural creases and within the hair as far as possible.

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2. **Lifting the skin:** The skin of the neck is gently separated from the muscle beneath; if needed, the superficial fat is thinned evenly.
3. **Removing deep fat:** The deep fat lying beneath the neck muscle (platysma), which cannot be reached by liposuction from the outside, is reduced in a controlled way.
4. **Digastric muscles and submandibular glands:** If needed, the front part of the digastric muscle that causes fullness under the chin is thinned. If a salivary gland (submandibular gland) is sagging and prominent, the protruding part may be partially reduced; the gland is not removed completely.
5. **Platysmaplasty:** The edges of the platysma muscle, which have separated in the middle of the neck and form bands, are stitched together to create an "internal corset"; if needed, the muscle is also suspended from the sides.
6. **Removing excess skin and closure:** The skin is redraped upwards and backwards without tension and the excess is removed. Bleeding is carefully controlled, a **drain** (a thin tube to remove fluid) is usually placed, the incisions are closed with fine stitches and a neck dressing is applied.

The planned components vary from person to person; not every step is performed in every patient. If a facelift is to be performed at the same time, a **separate consent form** will be signed for it.

ANAESTHESIA	APPROXIMATE DURATION	HOSPITAL STAY / DISCHARGE
General anaesthesia (in selected patients, sedation + local anaesthesia)	About 2–4 hours (longer with combined procedures)	Usually one night in hospital

3 Expected benefits and likelihood of success

- A more defined jawline and angle between the jaw and the neck
- Less fullness under the chin and fewer vertical neck bands
- Tightening of loose neck skin and a younger, fresher appearance

Most suitably selected patients are satisfied with the result. Most of the swelling settles in the first 4–6 weeks; the final result is seen within **6–12 months**. Results may be more limited in patients with poor skin quality, older age, major weight loss or previous neck surgery. About **5–10 in every 100** patients may need a small correction (touch-up) procedure.

4 Other treatment options

- **Not having surgery:** As this is a cosmetic procedure, you always have the option not to have it.
- **Submental (chin) liposuction alone:** May be enough for people with good skin elasticity whose fullness is due only to fat under the skin; it does not correct deep fat, muscles or glands.
- **Fat-dissolving injections (deoxycholic acid) or energy-based devices (radiofrequency, ultrasound):** Provide limited improvement in mild fullness and laxity; several sessions are usually needed.
- **Botulinum toxin to soften platysmal bands:** Reduces the bands temporarily (3–4 months); it does not correct sagging or fat.
- **A more limited neck lift:** Smaller operations that only remove excess skin or only include platysmaplasty; their effect is more limited.

5 What happens if the procedure is not performed?

If the operation is not performed, the current appearance of your neck will remain and natural ageing will continue. This does not pose a risk to your health.

You have the right to decline the recommended procedure, to postpone it, or to seek a second medical opinion. This decision will not affect any other healthcare offered to you in any way. You may withdraw your consent at any time before the procedure begins; please be aware, however, that once a procedure has started there may be stages at which stopping would be medically unsafe.

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6 Preparing for the procedure

- Blood tests and, if needed, an ECG and other investigations are performed before surgery; you will be assessed by the anaesthetist.
- **It is very important that your blood pressure is well controlled.** High blood pressure significantly increases the risk of a collection of blood in the neck (haematoma) after surgery. Ask the anaesthetist whether you should also take your blood pressure medicine on the morning of surgery (with a sip of water).
- Do not eat or drink anything for **6–8 hours** before surgery (follow the anaesthetist's instructions).
- Avoid strenuous exercise, saunas and heavy alcohol intake before surgery. Do not wear make-up, jewellery or necklaces on the day of surgery; bring a front-buttoning top that does not need to be pulled over your head.
- It is recommended that someone stays with you for the first 24–48 hours after surgery.
- Tell your doctor about **all medicines** you take (including blood thinners, aspirin, painkillers, weight-loss and hormone medicines) and any **herbal products and supplements** (such as omega-3, vitamin E, ginkgo, garlic, ginseng, fish oil). Do not stop any medicine on your own without your doctor's advice.
- Be sure to tell us about any **allergies**, bleeding or clotting problems, heart, lung, diabetes, blood pressure or thyroid conditions, previous operations and any problems you have had with anaesthesia.
- If there is any chance you may be pregnant, or if you are breastfeeding, tell us before the procedure.
- Smoking and nicotine products significantly impair wound healing. If possible, stop at least 2–4 weeks before the procedure and do not smoke during recovery.

7 Recovery: what to expect

First 24–48 hours	There will be a dressing and a drain on your neck. You will have swelling, tightness and discomfort when swallowing; pain is usually mild to moderate. It is important to sleep with your head raised (30–45°), not to bend your neck excessively and to avoid rises in blood pressure (no straining, keep any cough under control). The drain is usually removed on **day 1–2**.
Week 1	Swelling, and bruising **if it occurs**, are most noticeable at this stage and then start to settle. Numbness of the neck and under the chin is normal. The **neck garment/dressing** is worn day and night as advised. Stitches are removed on **days 5–10**.
Weeks 2–4	Most patients return to desk work and social life within 10–14 days. Firmness, a feeling of unevenness and tightness in the neck may continue. Strenuous exercise, bending, lifting and movements that strain the neck should be avoided for at least **3–4 weeks**.
Months 1–3	Remaining swelling and firmness gradually decrease. Numbness largely improves. Scars may be pink and slightly raised during this period.
Months 6–12	The final appearance is seen; scars fade and become finer. If a touch-up is considered necessary, it is usually done after at least 6 months.

8 Possible risks and complications

As with any medical procedure, this procedure carries some risks. Below, **expected temporary effects** and **complications** are listed separately and grouped by how often they occur. The figures given are **approximate** values based on the medical literature; your personal risk may be lower or higher depending on your age, general health, smoking and previous operations.

The great majority of complications are **temporary** and improve with appropriate treatment. Very rare but serious events are also included; being on this list does not mean they will happen to you, but you need to know about them to make an informed decision.

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Expected, temporary effects (not complications — a normal part of healing)

- Swelling of the neck and under the chin; bruising, **if it occurs**, settles within 1–3 weeks
- Temporary numbness of the neck skin and around the ears
- Tightness and firmness in the neck, and mild discomfort when swallowing
- Mild pain, itching and pink scars at the incision sites
- Unevenness and firm areas that can be felt under the skin in the first few weeks

Common conditions (most are temporary or resolve with treatment)

- **Prolonged swelling and firmness** — Firmness of the neck and under the chin may last several months; it improves with massage and time.
- **Minor asymmetry and contour irregularities** — Slight differences between the two sides of the neck or small dips/bumps under the skin may remain; most soften with time, some may need a small correction.
- **Seroma (fluid collection)** — A collection of clear fluid under the skin; usually treated by draining it with a needle in the clinic.
- **Need for a touch-up** — About **5–10 in every 100** patients may need a small correction for residual fullness, irregularity or scarring.

Uncommon conditions

- **Haematoma (collection of blood in the neck)** — Occurs in about **2–5 in every 100** patients; it is more common **in men and in people with high blood pressure** and usually appears within the first 24 hours. Small haematomas can be drained with a needle; large ones require a **return to the operating theatre** to remove the blood and stop the bleeding. A rapidly enlarging haematoma can **press on the windpipe and make breathing difficult**; this requires emergency treatment. A haematoma that is recognised late can impair the blood supply to the skin and lead to wound and scarring problems.
- **Temporary weakness of lower-lip movement (marginal mandibular / cervical branch of the facial nerve)** — In about **1–10 in every 100** patients, one side of the lower lip may look asymmetric when smiling. Most cases are due to stretching of the nerve or swelling and recover on their own within **a few weeks to a few months**.
- **Saliva leak / saliva collection (sialocele)** — In patients whose submandibular gland has been reduced, saliva may leak from the gland and collect under the skin. It usually resolves with repeated needle drainage, a pressure dressing and, if needed, medication; rarely a further procedure is needed.
- **Deformity from excessive tissue removal** — Removing too much fat or gland tissue can create a hollow in the middle of the neck, prominent muscles and a “skeletonised” look (“cobra neck” deformity). Correction may require fat grafting or further surgery; for this reason tissue removal is performed in a balanced and controlled way.
- **Noticeable or wide scars** — Scars under the chin and behind the ears may rarely thicken, widen or change colour. They can be treated with steroid injections, silicone gel or scar revision. If the incision behind the ear extends to the hairline, hair may thin along it. If incisions are made around the earlobe, the earlobe may rarely look pulled down and forwards; a minor correction can be made if needed.
- **Prolonged numbness** — Reduced sensation of the neck and around the earlobe may last several months; rarely it may be partly permanent.
- **Infection** — Occurs in **anywhere from fewer than 1 to 2 in every 100** patients. It usually settles with antibiotics; rarely drainage is needed.

Rare or very rare, but potentially serious conditions

- **Impaired skin blood supply and tissue loss (necrosis)** — Especially in **smokers**, people with diabetes and patients who develop a haematoma, wound breakdown and loss of skin at the edges may occur. Healing is prolonged, dressings and sometimes further surgery are needed, and a permanent scar may remain.

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- **Permanent nerve damage** — Permanent weakness of lower-lip movement occurs in **fewer than 1 in 100** patients. Very rarely the nerves that move the tongue (hypoglossal nerve) or give sensation to the tongue (lingual nerve), or the nerve supplying sensation to the earlobe (great auricular nerve), may be affected.
- **Airway problems and swallowing difficulty** — In very rare cases due to a large haematoma or marked swelling, the airway may need to be secured as an emergency, followed by monitoring in intensive care. Temporary difficulty swallowing may occur after the deep structures are corrected.
- **Injury to a blood vessel or salivary duct** — Bleeding from injury to the neck veins or problems with the salivary gland duct are very rare; they may require further treatment or a procedure.
- **Recurrent sagging** — Over the years the neck may relax again with ageing and weight changes; this is a natural process, not a complication. The neck muscle bands may also become visible again over time. Marked early laxity is rare.
- **Psychological dissatisfaction** — Despite a technically successful result, a person may find it difficult to adjust to their new appearance. Discussing expectations openly before surgery reduces this risk.

General risks that apply to any surgical / medical procedure

- **Anaesthesia risks:** If general anaesthesia or sedation is used, its risks (nausea, sore throat, dental damage, allergic reaction and, very rarely, heart or lung problems) will be explained separately by the anaesthetist, and your consent will be obtained on a **separate anaesthesia consent form**.
- **Bleeding and blood transfusion:** The need for a blood transfusion in these procedures is very rare. In the event of serious bleeding, blood or blood products may be needed; these carry their own very rare risks (such as allergy or infection).
- **Infection:** A wound infection after surgery is usually treated with antibiotics; rarely, drainage (letting the pus out) may be needed.
- **Blood clots in the leg veins (DVT) and in the lungs (pulmonary embolism):** Very rare in short head and neck operations. Walking early reduces the risk. Hormone medicines, smoking, obesity and a previous history of clots increase the risk.
- **Drug allergy:** An allergic reaction may develop to the medicines, antiseptics, tapes or suture materials used.
- **Scarring and healing problems:** How scars form varies from person to person; very rarely a thick (hypertrophic) scar or keloid may form.
- **Risk to life:** Death in planned head and neck surgery is extremely rare today; however, no surgical procedure is entirely free of risk. A serious complication may require treatment in intensive care and may result in permanent disability.

This list does not include every possible complication; other unforeseeable and very rare events may occur. If a complication develops, your doctor will plan the necessary treatment and follow-up; this may require additional treatment, medication, procedures or a further operation.

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9 When to contact us immediately

- **Rapidly increasing swelling, tightness or marked one-sided enlargement** of the neck, especially in the first 24–48 hours
- **Difficulty breathing, a change in your voice, inability to swallow** or a choking sensation (call 112 without delay)
- **Bleeding that does not stop** under the dressing, or blood increasing rapidly in the drain bottle
- **Fever above 38.5 °C**, increasing redness, pain, warmth or pus at the incision sites
- **Darkening, purple-black discolouration** of the skin, or the wound opening up
- Sudden severe pain, or pain that does not respond to painkillers
- Shortness of breath, chest pain, or one-sided swelling and pain in a leg

If any of these occur, call your doctor without delay: +90 288 502 50 20 · WhatsApp +90 555 333 21 20. If you cannot reach us, go to the nearest emergency department or call 112 (the emergency number in Türkiye and across Europe).

10 Medication and everyday-life advice

- For the first 2 weeks sleep on your back with your head raised; avoid bending your neck excessively, sudden head and neck movements and leaning forward.
- Wear the neck garment/dressing for the period advised (usually 1–3 weeks).
- Check your blood pressure regularly and do not miss your medicines; avoid straining, heavy lifting and uncontrolled coughing.
- Avoid strenuous exercise, saunas, Turkish baths and hot showers for the first 3–4 weeks.
- Protect your scars from the sun for at least 3–6 months.
- If you have travelled from abroad, flying is usually safe once the drain has been removed and the first check-ups are complete, generally from day 5–7.
- Take the medicines prescribed for you (antibiotics, painkillers, etc.) at the recommended dose and for the recommended time. Do not take aspirin or similar medicines that may increase bleeding without asking your doctor.
- On flights or journeys longer than 4 hours after surgery, drink plenty of water, move around regularly and wear compression stockings if advised; this reduces the risk of leg clots.
- During recovery you are advised not to drink alcohol and to avoid smoking and nicotine products.
- Please attend all follow-up appointments. If you have travelled from abroad, follow-up after your return can continue by online consultation and photos/videos; if you need to see a local doctor, you will be given a written operation report and our recommendations.

11 Unexpected findings during the procedure, tissue samples and blood products

If an unforeseen situation arises during the procedure, your doctor will stay within the scope of the procedure you have consented to on this form. **Unless there is a risk to life or of serious harm to an organ**, no additional procedure that you have not consented to will be performed; in such a case the procedure will be brought to a close at a safe point, and you will be informed afterwards so that the decision can be made together with you. If your life is in danger, the emergency treatment needed to protect your life or the organ will be carried out.

Tissues removed during the procedure will be sent for **pathological examination** where necessary to confirm the diagnosis. Additional treatment or follow-up may be needed depending on the result.

Blood transfusion is very rarely needed in these procedures and will only be given when it is vital. If you do not accept blood or blood products (for example for religious reasons), you must tell us before the procedure; the possible consequences of this decision will be discussed with you separately and recorded under “Information specific to you”.

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12 Photographs, image records and your personal data

Photographs/videos may be taken before and after the procedure for planning, follow-up and assessment of the result. These records form part of your medical file and are kept confidential. Your images will **never be used for advertising, promotion or social media.**

Your images may be used for any other purpose **only with your separate and explicit permission**. Declining will not affect your treatment in any way, and you may withdraw your permission at any time:

- ☐ I agree that my images are used **only for medical records and treatment follow-up**.
- ☐ I permit my images to be used for scientific publication and medical education **in a way that does not identify me** (face/eye area covered).
- ☐ I permit my images to be used for scientific publication and medical education **in a way that may identify me**.
- ☐ I **do not permit** scientific/educational use.

Your personal and health data are processed under the Turkish Personal Data Protection Law No. 6698 (KVKK) and the relevant health legislation, for the purposes of diagnosis, treatment and legal obligations, by persons bound by professional confidentiality. You may consult our Data Protection Information Notice for details.

13 Information and notes specific to you (completed by the doctor)

Patient-specific additional risks (co-existing diseases, medicines, previous operations), details of the plan, the patient's expectations, questions asked and answers given. If there is nothing to add, write "none".

14 Patient declaration

- ☐ I have read and understood this form in my own language. I was given enough time to think; I had the opportunity to ask questions and received satisfactory answers.
- ☐ My condition, the aim of the recommended procedure, how it will be done, by whom and where, its approximate duration, expected benefits, likelihood of success, alternatives and what may happen if it is not done have been explained to me verbally and in writing.
- ☐ The common and potentially serious risks, and the possible consequences of these complications (including that further treatment or a repeat procedure/operation may be needed), have been explained to me. I understand that the outcome of any medical procedure **cannot be 100% guaranteed**.
- ☐ I have given accurate and complete information about my health, the medicines I take, my allergies and past illnesses. I understand that incomplete or incorrect information may increase the risks.
- ☐ I understand that following the instructions given before and after the procedure plays a part in the result, and that attending follow-up appointments is important.
- ☐ I know that I can withdraw my consent at any time before the procedure begins. I understand that signing this form does not remove the doctor's duty of care.
- ☐ I am making this decision of my own free will, without pressure from anyone.
- ☐ I understand that the operation does not stop ageing, that the neck may relax again over time, and that the result is limited by my skin quality, weight and bone structure.
- ☐ I understand that keeping my blood pressure under control and following the post-operative instructions are important to reduce the risk of bleeding (haematoma).

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☐ I understand that this procedure is **cosmetic**, that it is not medically necessary and that I always have the option not to have it. I have discussed my expectations with my doctor, and the realistic result that can be achieved has been explained to me.

☐ I understand that perfect symmetry or an exact copy of the appearance in a photograph cannot be guaranteed, that the final result depends on how my tissues heal, and that in some cases a correction procedure may be needed.

☐ **I GIVE MY CONSENT to the procedure described above.**

☐ **I DO NOT ACCEPT the procedure described above. The possible consequences of refusing have been explained to me.**

We recommend the patient writes in their own handwriting: "I have read and understood this form and I give my consent."

15 Signatures

	FULL NAME	SIGNATURE	DATE / TIME
Patient			
Legal representative (parent / guardian) — relationship:			
Doctor providing information and performing the procedure			
Interpreter (if needed) — language:			
Witness (if needed)			

Interpreter's statement: I have translated this form and the doctor's verbal explanations to the patient / legal representative completely and accurately, in a language they understand.

FOR PATIENTS UNDER 18 OR WITH LIMITED DECISION-MAKING CAPACITY

For patients under 18, consent is given by a **parent or legal guardian**. The child is informed in a way appropriate to their age and maturity and their view is sought. For patients lacking capacity, consent is given by the legal representative.

☐ If we share parental responsibility, I declare that the other parent is aware of and agrees to this procedure.

☐ I declare that there is no court order or dispute concerning custody / guardianship (if there is, the relevant document has been shown to the doctor).

The child was informed in an age-appropriate way; the child's view: _____

This form has been prepared in two copies: one is kept in the patient's file and one is given to the patient / legal representative.

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WITHDRAWAL OF CONSENT (COMPLETE ONLY IF YOU CHANGE YOUR MIND)

I withdraw the consent I gave above. The possible consequences of withdrawing have been explained to me.

Name, signature, date and time of patient/representative withdrawing consent: _____

This form has been prepared in accordance with the Turkish Patient Rights Regulation and the informed consent principles of the Turkish Medical Association. It complements, but does not replace, the verbal explanation given by your doctor.